



Title I Schoolwide Program Plan Template

Any school that operates a Title I Schoolwide Program must, with the input of stakeholders, conduct a comprehensive needs assessment (CNA) and, based on the analyses of the CNA, develop a schoolwide plan. This template or a separate Title I Plan is not required if a school's improvement plan contains all required components of the Elementary and Secondary Education Act (ESEA).

School/LEA Information

School Name: West Elementary School

LEA Name: Tooele County School District

Plan for the School Year : 2025-2026

A. Stakeholder Planning Team – ESEA Sec. 1114(b)(2)

1. List the stakeholders who developed, and will help implement and evaluate, the Schoolwide Program (add extra lines for any additional stakeholders).

Stakeholder Title	Stakeholder Name	Stakeholder Signature
Principal/Director	Thomas Trotter	
LEA Title I Director (required)	Stephanie Rowley	
Faculty Member	Aly Larsen	
Faculty Member	Emilee Liddiard	
Faculty Member	Kellie Thomas	
Faculty Member	Stephanie Fenton	
Faculty Member	Sydney Whatcott	
Parent, non-school employee	Kristina Hansen	
Parent, non-school employee	Brooke Bevan	
Parent, non-school employee	Brian Hadzik	
Community Member, not associated with school	Terra Sherwood	
Paraprofessional	Deanna Young	
PTA President	Jennifer Andrews	

2. Describe the process for involving stakeholders and how their input was used to develop the Schoolwide Program.

- Our school team put together a presentation of our comprehensive needs assessment and presented it to the staff, the community council, and to the school board and district leadership. The data was shared, it was discussed where we have been successful and where we need to improve, and time for questions and comments was provided.

B. Comprehensive Needs Assessment (CNA) – ESEA Sec. 1114(b)(6)

The CNA (Comprehensive Needs Assessment) is a systematic effort to acquire an accurate and thorough picture of the strengths and weaknesses of the school that impact equitable student outcomes.

1. List all sources of data collected and analyzed (i.e., longitudinal student academic and nonacademic achievement, curriculum and instruction, professional learning opportunities, school climate and culture, family and community engagement, etc.)

- School vision statement
- PBIS Initiatives
- The Happiness Advantage staff book study
- Faculty, parent, and student stakeholder surveys
- Safety and building improvements
- Semester plan goals
- Acadience reading data
- Acadience reading pathways of progress
- Observation data
- Student data folders
- Staff utilization
- ATSI Designation – Economically disadvantaged students data
- Multilingual student data
- Acadience math data
- RISE math data – yearly and longitudinally
- RISE science data – yearly and longitudinally
- AAPPL German proficiency data
- Attendance data
- Educator's Handbook behavior data

2. Summarize main findings including an examination of student, teacher, school, and family/community strengths and needs. Findings should include detailed analysis of all student groups of 10 or more, i.e., students identified as economically disadvantaged, students with disabilities, students identified as English learners, and students by major racial and ethnic groups. **This analysis should show direct relationships from the data and root causes in the Comprehensive Needs Assessment to the priorities** identified in the next section.

Successes:

- The Happiness Advantage improved moral throughout the school
- Reading scores improved in all grade levels
- Attendance was 94.6% for the year
- Behavior data constantly decreased all year
- District awards for reading improvement

Areas Needing Improvement

- Economically Disadvantaged ATSI designation
- Math scores are well below proficient
- Family and parent involvement is low
- Building safety improvements are needed
- Training is needed to improve PLC meeting effectiveness
- Science data is pretty stagnant

3. Prioritize the school's top needs as evidenced by the CNA.

- Priority 1: Our school is designated an ATSI school for our economically disadvantaged student population. We have had this designation for the last 4 years now. It is essential that we target this group of students to help them succeed, to help them build confidence, to overcome this ATSI designation, and to show our economically disadvantaged students their potential.
- Priority 2: Our student's math scores have shown a lack of appropriate growth on both Acadience and RISE tests. A greater focus on tier 1 math instruction and regular math interventions need to happen. Teachers also need to become more familiar with the math standards and math curriculum and to appropriately adjust instruction based on data through the PLC process.
- Priority 3: Our parents and families have had very few opportunities to be in our building and to be involved. We need to change this, so that parents and families feel a part of our school community. Through events focused on academics, climate and culture, positivity, involvement, and belonging, we will be able to grow our community outside the walls of the school. This will also help parents to understand the academics and other needs of our school.

C. Coordination and Integration of Services and Resources – ESEA Sec. 1114(b)(5)

Show how Title I funds, along with other local, state and/or federal resources, will be used to implement the Schoolwide Program strategies.

<u>Schoolwide Program Initiatives</u>	<u>Funding Stream/Program*</u> <i>(list all that apply)</i>	<u>Approximate \$ Amount</u>	<u>Additional Resources**</u>
<u>Initiative:</u> <u>Instructional aides will be hired to help with tier 1 reading and math instruction and interventions and to allow teachers appropriate time to meet in PLCs and address needs.</u>	• Title I, Part A • TSSA • Trustlands • At Risk Funds	<u>\$119,000</u> <u>(\$30,989.74 from Title 1)</u>	<u>**</u> <u>Teacher quality, academic rigor, instructional time and attention, early learning/interventions.</u>
<u>Initiative:</u> <u>After school classes for economically disadvantaged students to promote academics, social and emotional skills, and behavior</u>	• Title I, Part A • Local school funds	<u>\$7,500</u> <u>(\$6,947.59 from Title 1)</u>	<u>**</u> <u>Academic rigor, instructional time and attention, early learning/interventions, diverse and inclusive school approaches.</u>
<u>Initiative:</u> <u>Morning and evening events focused on academics, climate and culture, parent and family involvement, and positivity will take place throughout the year.</u>	• Title I, Part A • Building Leadership Stipend Money • Local school funds • PTA Funds	<u>\$5,000</u> <u>(\$2,396.51 from Title 1)</u>	<u>**</u> <u>Academic rigor, instructional time and attention, early learning/interventions, diverse and inclusive school approaches, family academic engagement.</u>

***Additional Programs/Funding Streams:** Title I, Part C, Education of Migratory Children; Title I, Part D, Programs for Children and Youth Who are Neglected, Delinquent, At-Risk; Title II, High Quality Teachers and Principals; Title III, Language Instruction for Limited English Proficient and Immigrant Students; Title VI, Indian Education; Title VII, Part B, McKinney-Vento, Education for Homeless Children and Youths; IDEA, Part B, Assistance for Children with Disabilities; Carl D. Perkins, Vocational and Technical Education (CTE); Title I, Family Engagement; Title I, Sec. 1003(a) and (g), School Improvement; State School Turnaround; Utah Trust Lands; State Enhancement for At-Risk Students (EARS); etc.

****Additional Equity Resources:** teacher quality, school leadership quality, academic rigor, instructional time and attention, early learning/interventions, whole child approaches, diverse and

inclusive school approaches, family academic engagement, etc.

Moorside Medical Practice

Complaints Procedure

~~Comments, Complaints and Suggestions~~

~~Our aim is to provide the highest level of care for all our patients. We will always be willing to hear if there is any way that you think that we can improve the service we provide.~~

~~We always try to give you the best possible service, but there may be occasions when you feel this has not happened. This leaflet explains what should do if you have a complaint about the service we provide for you.~~

~~If you have any complaints or concerns about the service that you have received from the doctors or staff working for this practice, please let us know.~~

Making a Complaint

~~We hope that most problems can be sorted out easily and quickly, often at the time they arise and with the person concerned.~~

~~If your problem cannot be sorted out in this way and you wish to make a complaint, we would like you to let us know **as soon as possible**—ideally within a matter of days or at most a few weeks—because this will enable us to establish what happened more easily.~~

~~If it is not possible to do that, please let us have details of your complaint:~~

- ~~• Within 12 months of the incident that caused the problem; or~~
- ~~• Within 12 months of discovering that you have a problem, provided that is within 12 months of the incident.~~

~~The Practice Manager will be pleased to deal with any complaint. She will explain the procedure to you and make sure that your concerns are dealt with promptly. You can make your complaint:~~

~~In person~~—ask to speak to the Business Manager Debra Pryce.

~~In writing~~—some complaints may be easier to explain in writing—please give as much information as can, then send your complaint to the practice for the attention of the Practice Manager as soon as possible.

What We Shall Do

~~Our complaints procedure is designed to make sure that we settle any complaints as quickly as possible.~~

~~We will acknowledge receipt within 3 working days, and aim to have looked into the matter within 10 working days. You may then receive a formal reply in writing, or you may be invited to meet with the person(s) concerned to attempt to resolve the issue. If the matter is likely to take longer than this we will let you know, and keep you informed as the investigation progresses.~~

When looking into a complaint we attempt to see what happened and why, to see if there is something we can learn from this, and make it possible for you to discuss the issue with those involved if you would like to do so.

When the investigations are complete your complaint will be determined and a final response sent to you.

Where your complaint involves more than one organisation (e.g. social services) we will liaise with that organisation so that you receive one coordinated reply. We may need your consent to do this. Where your complaint has been sent initially to an incorrect organisation, we may seek your consent to forward this to the correct person to deal with.

The final response letter will include details of the result of your complaint and also your right to escalate the matter further if you remain dissatisfied with the response.

Complaining on behalf of someone else

Please note that we keep strictly to the rules of medical confidentiality.

If you wish to make a complaint and are not the patient involved, we will require the written consent of the patient to confirm that they are unhappy with their treatment and that we can deal with someone else about it.

Please ask at reception for a third party consent form which the patient will need to sign to enable the complaint to proceed.

Where the patient is incapable of providing consent due to illness or accident it may still be possible to deal with the complaint. Please provide the precise details of the circumstances which prevent this in your covering letter.

Please note that we are unable to discuss any issue relating to someone else without their express permission, which must be in writing, unless the circumstances above apply.

We may still need to correspond direct with the patient, or may be able to deal direct with the third party, and this depends on the wording of the authority provided.

What You Can Do Next

We hope that, if you have a problem, you will use our procedure. We believe that this will give us the best chance of putting right whatever has gone wrong and the opportunity to improve our practice.

If you are dissatisfied with the response to your complaint, you can approach the Health Service Ombudsman. They are completely independent of the NHS & Government. The contact details are:

The Parliamentary and Health Service Ombudsman
Millbank Tower
Millbank
London
SW1P 4QP

Tel: 0345 015 4033

Email: phso.enquiries@ombudsman.org.uk

Website: www.ombudsman.org.uk

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If you need assistance in making your complaint or you are not happy to talk to the practice you may contact the Complaints Department at NHS England:

NHS England
PO Box 16738
Redditch
B97 9PT
Tel: 0300 311 2233

Email: England.contactus@nhs.net

Free help and advice can be obtained from the PALS

Call 0161 778 5665 between 09.30 and 16.30 Monday to Friday or drop into one of the front of house PALS offices at your nearest Northern Care Alliance hospital.

The Independent Complaints Advocacy (ICA) can also provide free independent advice and support to people making complaints:

Tel: 08088023000

Email: hub@carersfederation.co.uk

Help us get it right

We constantly try to improve the service we offer to our patients.

Please let us know when you think we have done something well or if you have any suggestions as to how we can do something better.

Please send any comments you may to:

Debra Pryce Business Manager
Moorside Medical Practice
681 Ripponden Road
Oldham
OL1 4JU
Tel: 0161 357 5250